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**WHY FIGHTING GENDER-
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INTRODUCTION



Gender-based violence is a worldwide menace that has attracted lots of reportage. It is a huge issue that cannot be underestimated, considering its grave impact on primary females and the larger society. Millions suffer from this act. The World Health Organisation (WHO) has posited that one of them suffers greatly from psychological or physical violence from a spouse or an intimate partner (UN Women, 2017).

Considering the various forms of violence meted out to women, it is crucial to consider the reasons behind them and know how these women are disadvantaged. The term gender-based violence (GBV) brings to bear a broader emphasis on the various kinds of violence that women suffer. GBV range from sexual harm to psychological harm as well as physical and total discomfort to women. Other concerns that are considered uncomfortable include threats that result in the deprivation of liberty and coercion to do things against their will. Some researchers have added that other notable occurrences emanate from GBV such as an increase in the risk of a number of health issues, which are considered public health concerns over the years (Gelaye et al., 2009; Philpart et al., 2009).

Other detrimental effects of GBV are physical injury and mental health outcomes, which include low self-worth and increment in unwanted pregnancy, maternal mortality and sexually transmitted infections (STIs), predominantly HIV (Gelaye et al., 2009). In addition, the probability of women who experience sexual and or physical violence is higher, almost 1.5 times than women who do not experience this form of violence. (UN Women, 2017).

The university is an institution of higher learning that produces skills for the job market and society (Vicente-Molina, 2013). Gender-based violence exists in our universities locally and internationally. In Africa, gender-based violence is a thorny but precarious concern that must be accorded some urgency since violence cases are on the rise across many countries on the continent (Dunkle et al., 2004). Students in such institutions are mainly young adults, not more than 25 years. Many university students in Africa are brought up in typically patriarchal cultures, where their gender notion may be in direct contradiction to those accepted in tertiary institutions (Bawa, 2012).

Training students occur in culturally diverse and multi-ethnic societies where students from varying socio-demographic and socio-economic backgrounds come together in search of knowledge.

School-related gender-based violence in higher institutions represents a serious blockade to learning and training, which ultimately leads to physical harm and instigates severe psychological and educational adverse consequences, which must be given utmost attention (Roskin – Frazee, 2020).

Gender-Based Violence (GBV) in institutions of higher learning

Considering the climate of tertiary institutions, one would not hesitate to emphasize gender-based violence. These institutions are male-dominated and instances of rape, theft, insults, threats are almost always occurring on these campuses to the detriment of females who are mostly at the receiving end (Kaufman et al., 2019). In a research done by WHO, it emerged that schools and universities were exceptionally susceptible to GBV (Gordon & Collins, 2013). Unfortunately, this problem is not well tackled in our tertiary institutions (Gordon & Collins, 2013).

Females on university campuses are likely to experience GBV one time or another due to their minority status. Over the years, universities and other tertiary institutions in Ghana recorded a higher intake of males than females, which is a cause of the domineering attitude amongst the males over the females (Morley, 2011). However, other factors account for GBV aside from the male to women ratio. In Africa, patriarchal traditions have had an unbearable influence on violence amongst women (Bawa, 2012). In most African societies, the man should be the final authority in the home, workplace and family gathering. However, the upbringing of the male child is quite different; male children are socialized to rule over the women (Morley, 2011). This understanding translates into violence against women who submit to male authority. Men replicate this attitude in the universities, which elicit fear, insecurity, and other psychological issues on the part of women.

Some young males in the university engage in illicit drugs, which increases their propensity to engage in gender-based violence against women (Salakhova et al., 2019). In addition, tertiary institutions, especially universities, are places for experimentation. This is because there is freedom for exhibiting certain characters by students, which were not allowed in their previous academic settings, such as the senior high schools (Salakhova et al., 2019). However, this newfound freedom can also be a challenge for females since males tend to abuse privileges given them (Morley, 2011).

Sexual violence, harassment and verbal threats are birthed because of some of the factors mentioned above, creating an unfavourable atmosphere for females in institutions of higher learning.





The World Health Organization, in multi-country research on violence meted out to women, found that the lifetime and present prevalence of physical and sexual intimate partner violence span from 15 to 71% and 4 to 54%, respectively, with that of emotional violence, pegged between 20 to 75% (Garcia - Moreno et al., 2005).

A different research work conducted by the World Health Organization (WHO) approximates that the lifetime prevalence of intimate partner violence among female youths aged 15-19 was 29.4 and 31.6% for ages 20-24. The most significant prevalence of intimate partner violence was reported in the African region, notably in Sub-Saharan Africa (65.64%) (WHO, 2013). In addition, evidence from Sub-Saharan Africa (SSA) region revealed increased rates of GBV in educational institutions. The Global Based School Survey (GBSS) results showed that the extent of current physical and sexual violence in five African countries ranged from 27-50% and 9-33%, respectively (Brown et al., 2009).

In a study undertaken in South Africa among adolescents aged 10-17 years, it was found that the lifetime prevalence of physical abuse was 56.3% (18.2%), emotional abuse 35.5% (12.1%), and sexual abuse 9% (5.3%) (Meinck et al., 2016). In addition, researchers showed that the prevalence of attempted rape (18.7%), actual rape (23.4%), physically violent harassment (8.7%), verbal harassment (24.2%), and forced sexual initiation (11.2%) among female students in an institution of higher learning which was a university (Tora et al., 2013). It should be noted that, over the last two decades, a significant amount of research on GBV has been carried out and published in SSA.

However, the current works have not found the prevalence of GBV among female youths in tertiary schools and should be well looked at (WHO, 2010).

Looking at gender-based violence through the lens of World Organisations



WHO

According to the World Health Organisation (WHO), the violence meted out to women, and the consequences are not new. The effects span physical, mental and reproductive health (WHO, 2013). GBV tramples upon the rights of the vulnerable in society, including women and girls, and denies some women adequate health and general wellness. The WHO asserts that it is a public health issue that affects about 33% of women around the globe (WHO, 2013).

Findings from WHO's research on global and regional estimates revealed that about 35% of women globally have one time or the other been involved with either physical and/or sexual intimate partner violence or non-partner sexual violence. Thus, even though many types of violence happen to women, this shows that a significant fraction of women are affected (WHO, 2013).

The WHO also asserts that many cases concerning gender-based violence are associated with intimate partner violence. WHO in 2013 showed that, for all women who have been in relationships, about 30% are abused by their partners. The research conducted by WHO and the London School of Hygiene mentioned the rise in murder cases to about 38% globally due to intimate-partner relationship (WHO, 2013). In addition, women who experience gender-based violence are prone to abortion, depression, and, in certain instances, HIV. This assertion was made when those women who are GBV free were compared to those that suffer it (WHO, 2013).



UNICEF

UNICEF has been emphatic on their stance on GBV, stating that it is a deplorable way of depriving girls and women of their fundamental human rights and has declared it a public health issue, which needs to be tackled.

The organization is significantly involved in solving issues relating to GBV, considered emergencies and have strategies that have worked very well, especially in parts of Africa (UNICEF, 2014). UNICEF's priority when it comes to GBV has been advocating and protecting the rights of children and women to fulfil their full potential and good health (UNICEF, 2014).

The agency posits that many girls under age 18 have had sexual acts committed against their will and have so far helped millions with their interventions globally (UNICEF., 2014). An approximated number of over 1 million women and girls and boys were helped through their interventions (UNICEF, 2018).

UNICEF is hailed worldwide for helping significantly prevent gender-based violence in emergencies. They use three methods; supporting women and children who experience GBV, decreasing the risk of GBV across other humanitarian sectors, and curbing GBV by addressing its underlining conditions.



USAID

The United States of America is overly committed to curbing GBV worldwide, no matter the form that GBV takes. USAID considers GBV a considerable violation of human rights and a big problem to public health. It is also considered a hindrance to many activities that include political, civic and economic engagements (USAID, 2012).

USAID programs seek to tackle the fundamental factors that spring up violence and helps to do away with the root causes. The agency is committed to solving the economic and health needs of people that suffer from GBV. The USAID puts in place strategies that prevent and protect affected individuals to help provide for the needs of victims around the globe.

The organization posits that globally, women fall victims to intimate partner violence and is the usual form of violence reported mainly by women (USAID, 2012). In 2012, 150 million girls suffered from forced sexual intercourse and other GBV types, which is likely to increase in the coming years (USAID, 2012).

USAID is guided by certain principles of commitment and prompt response to GBV worldwide, setting up goals that are achievable and sustainable and implementation and monitoring processes that can help the agency solve the issue of GBV (USAID, 2012).



Commonly meted out GBV in our universities

Considering how crucial women's education is, especially in higher learning institutions, perceptions on GBV must be critically assessed, considering the persisting disparities between males and females regarding inequality in gender.

Some researchers have hinted that GBV is a never-ending event in tertiary institutions. However, Osuna – Rodriguez et al., (2020) research revealed that there are varied perceptions when it comes to GBV. Some perceptions about GBV stem from our culture in Africa, where there are patriarchal definitions of who a man is and his roles.

Some people think it is a prerequisite for low self-esteem and poor academic performance (Vázquez, Torres & Otero, 2012). This assertion is true and usually accompanies humiliation, especially those at the situation's receiving end. At any point in time, self-esteem plays a pivotal role in the overall inner joy that a person has. Unfortunately, GBV takes that joy and pride of being a woman, cutting back her confidence to partake in some universities such as journal club meetings, SRC positions etc.

In the university setting, the male population is already enough to scare women away or make them think lowly of themselves. In almost all universities in Ghana, the male population outnumbered the female population (Vázquez, Torres & Otero, 2012).

In an era where women are more empowered to do wonderful things for themselves and the country, it is necessary to curb all negative perceptions of GBV to enable them to bring out their full potential and enhance their capacity.

One of the most talked-about GBV on university campuses is intimate partner violence (IPV) (Umana, Fawole, Adeoye, 2014). The magnitude of this problem has been associated with males, though females can also be violent. Research has proven that males are significant culprits of most cases of intimate partner violence. However, that not being enough; some research works have concluded that violence that is considered physical disturbs emotional stability and leads to psychological abuse and primarily sexual assault. These acts are carried out by the perpetrators repeatedly. WHO asserts that women who go through this violence allude to the fact that emotional and psychological instability are the worst things that happen to them (WHO, 2013).

Forced prostitution is a growing GBV on our university campuses (Watts & Zimmerman, 2002). This behaviour is traceable to some factors. For some, while on campus, their boy-friends force them to engage in the act to get money. Others also come to campus financially destabilized with no help from parents to help survive the semester (Iliyasu et al., 2011). This makes it quite difficult for some students to survive and creates a huge vacuum, which some girls fill by practising prostitution to make ends meet.

Verbal or emotional violence is also one of the main types of GBV meted out to students, especially females (Iliyasu et al., 2011). Usually, men take advantage of their physique and strength to threaten and or insult women on campuses for some misunderstandings that arise. Usually, lecturers play a part by calling ladies names because they cannot answer questions right and conduct themselves in a manner the lecturer wanted. This has a severe effect on self-worth and the level of confidence among women. Therefore, GBV of this form should not be encouraged as women are supposed to express themselves freely without fear to bring out the best in themselves.

GBV is one of the major causes of poor psychological health worldwide (Heise, Ellsberg & Gottmoeller, 2002). Moreover, women at the receiving end of GBV suffer from emotional abuse, which does not augur well. WHO estimates millions of people suffering from this menace (WHO, 2013). Psychological abuse is defined as 'controlling and coercive behaviour, including isolating romantic partners from others; denigrating and dominating them; and using recurring criticism, threats, and verbal aggression' (O'Leary, 1999).

The state of mental health deteriorates when GBV is involved. In addition, for students who engage in intimate relationships on campus, many females are affected one way or the other by the action of verbal violence, physical violence, sexual coercion and forced prostitution.

Students threaten suicide and other unthinkable acts because of these GBV meted out to them. Students are depressed, which also triggers anorexia and other medical conditions that are not favourable. Many rape victims go through much psychological trauma for the rest of their lives if the proper medication and clinical advice are not sought. The adverse health consequences of GBV are a lot and demand that the proper measures are in place to salvage the situation.



Rape as major GBV in the universities

University campuses are plagued with sexual assaults, which is now a rising concern for public health. Therefore, it has become necessary to explore matters exclusive to universities that contribute significantly to issues of sexual assault and the perceptions of people about the situation. In addition, research has shown that a proportion of about 20%–26% of women tends to become victims of sexual assault in their lifetime (Cantor et al., 2015; Fisher, Cullen, & Turner, 2000). Therefore, it is essential to comprehend the different factors that cause the low reportage of rape on campuses, particularly by women, who are most susceptible to victimization (Boateng, 2015).

In Ghana, rape is illegal and has a law that backs it, stating that 'whoever commits rape shall be guilty of the first-degree felony and shall be liable on conviction to imprisonment (Act 29 of Ghana, Section 97). Although this law exists, improper sexual behaviours and violence constitute a regularly occurring issue – with rape undoubtedly remains a substantial social difficulty in Ghana (Quarshie, Osafo, Akotia, Peprah, & Andoh-Arthur, 2017).

Statistics at the department of the Domestic Violence and Victim Support Unit (DOVVSU), in collaboration with the Police Service of Ghana, stated in their reports that they had a lot of cases about rape, incest and defilement (Ghana Police Service, 2015) during a 15 – year review from 1999 to 2014.

The universities represent an enlightenment space, and students and staff are to comport themselves to speak well of them. Yet, unfortunately, rape cases committed solely or by a gang are on the rise in many university campuses, with Ghana being no exception. Though victims shy away from reporting some of these issues, it is public knowledge that Ghana's universities record a high incidence of rape cases either by an intimate partner or by others (Kelly, McGarr, Lehane, & Erduran, 2019).

The public demands specific functions of the police and other NGOs committed to this course, especially in Africa and Ghana. For instance, there should be a direct collaboration with the media organizations to increase public safety campaigns on measures to curb rape, paying more attention to girls and women. Guardians and or parents and lecturers can continue with these safety campaigns and increased education in their various houses and the lecture theatre, respectively.

Universities should consider investing in their counselling services to help victims who suffer this deadly act psychologically fit and free from guilt and shame.

Impact of GBV on reproductive health

Violence against women tends to have many unfortunate results on reproductive health, as suggested by research (Peacock & Levack, 2004). Physical violence, as well as sexual abuse, tends to cause diseases and pregnancies that are unwanted. Unfortunately, some women are coerced into having sex. Therefore, the effects of GBV could escalate and breed some devastating consequences. This happens when women resist sex or appeal for contraceptive use (Peacock & Levack, 2004).

Gynaecological disorders are usually the disturbing characteristics women go through. In addition, some studies have revealed that women who go through GBV are likely to suffer from chronic pelvic pain, especially if there is a history of consistent sexual abuse (Peacock & Levack, 2004).

Students at the various universities who agree to keep pregnancies have issues with pregnancy outcomes, especially if GBV continues. Health consequences that emanate from violence during pregnancy are enormous, and the effects are significantly felt; by both the woman and the baby (Coast, Leone & Malviya, 2012). It emerged prenatal care might be stopped when violence continues. Such women are at a higher risk of infections of the reproductive system as well as miscarriages. (Heise, 1994).

Implications of gender-based violence on academic performance

Studies have found academic performance as one of the pivotal issues marred by GBV. Although primary research on this subject has not extensively focused on women, many studies now have looked at GBV concerning academic prowess among tertiary students.

For instance, careful consideration of the situation shows that physical Intimate Partner Violence (IPV) has been implicated as a culprit in hindering school output, mainly when cumulative GPA is used as a standard for assessing students' performance (Mengo & Black, 2016; Tamsen & Butler-Barnes, 2016). Studies also show that GBV generally affects the GPA negatively (Mengo & Black, 2016).

Further research also stipulates that physical and sexual IPV contribute to school dropouts in colleges, with IPV being the primary cause (Mengo & Black, 2016).

GBV harms the victim's performance academically. However, not much is considered as factors that facilitate this finding. There is a strong relationship between GBV and academic output (Wood et al., 2020).

Gender-based violence and the SDGs

One hundred ninety-three nations are members of the United Nations (UN). During a conference in September 2015, these member states considered a set of goals known as the Sustainable Development Goals (SDGs). These goals are a modification and improvement to its predecessor the, Millennium Development Goals (MDGs). The conference accepted that consistent violence against women for more than a decade in the era of MDGs had halted the growth of the MDGs, considering the disturbing consequences on the inability of women to contribute to and benefit from broader developmental processes (WHO, 2016). WHO asserts that about 30% of women around the globe have been through physical and or sexual violence (WHO, 2016).

This ensured that the SDGs curbed that the rate of violence meted to women and girls became paramount. In addition, the SDGs confirmed that there are vibrant leadership and advocacy actions that are good contributors to solving the issue in providing exemplary financial commitment and the needed motivation.

Among the SDGs, the fifth one carefully considers making gender equality and women empowerment a reality. It seeks to prevent all disturbing practices and violent attacks on women and girls. This goal aims to bring to the minimum violence and loss of lives among women.

In formulating the SDGs, the UN conference hinted that the extent of violence women suffer is alarming but insisted that the menace is curbed. This is a significant acknowledgement of the campaigns against gender-based violence (GBV).

Some Empirical evidence on GBV

Several researchers have conducted studies on the subject and have come up with some evidence to buttress the point that has been strongly made in this study.

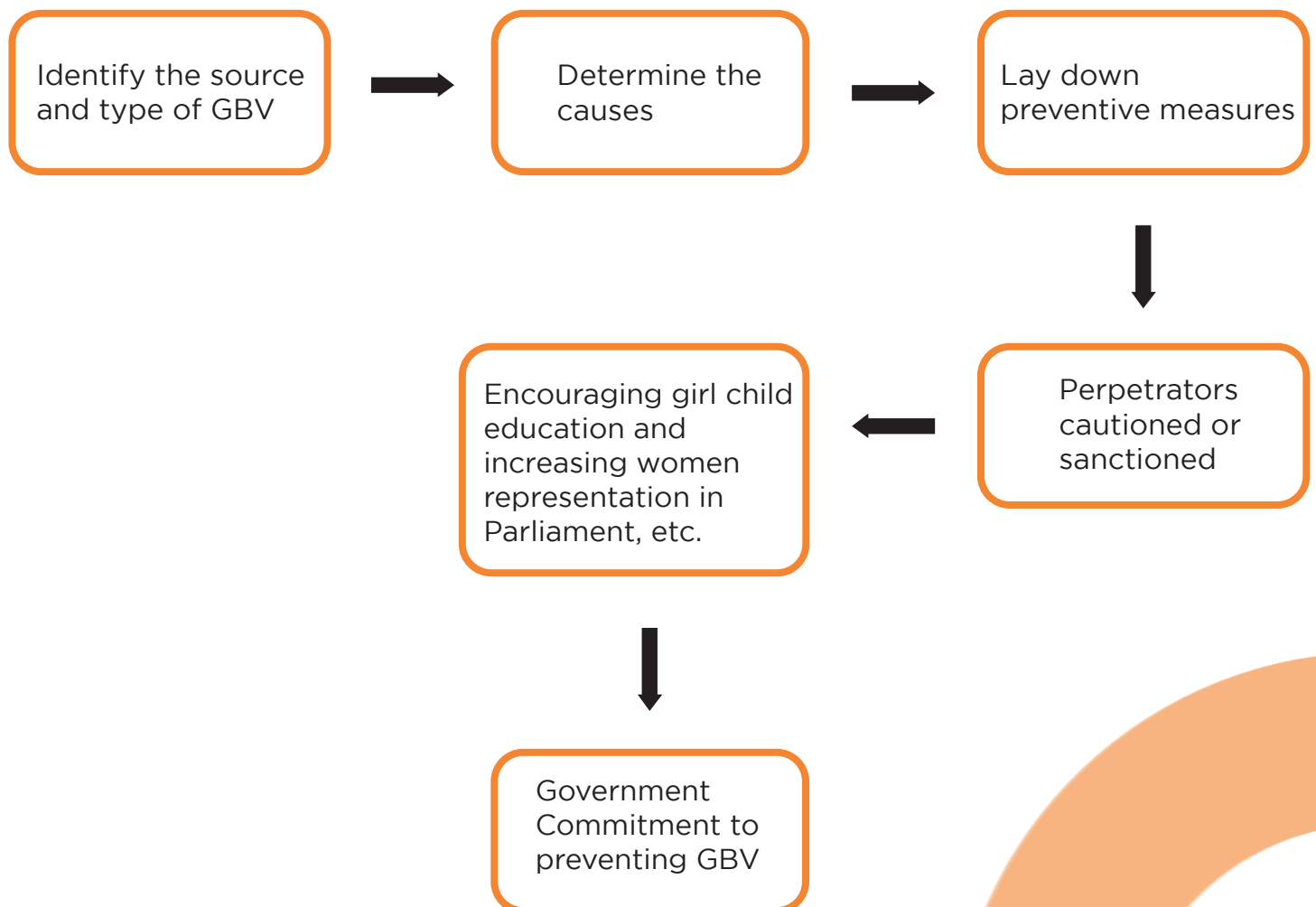
Fawole, Balogun & Olaleye (2018) used a comparative cross-sectional study design in public and private schools at Kwara State with 640 students. At least one form of GBV was experienced by 89.1% of public 84.8% private school students. The most typical type of GBV encountered was psychological abuse. Norman, Aikins & Binka (2013) investigated the prevalence and incidence of traditional and contra power sexual harassment in medical schools in Ghanaian universities in a cross-sectional study using 409 medical students. Their research revealed that women are highly susceptible to sexual abuse as compared to men. They also showed one of the factors that gravely affects health is sexual abuse and harassment. The authors found that sexual harassment is not new in tertiary institutions, especially when a student usually needs some academic help from colleagues or tutors.

Iliyasu et al. (2011) worked on the prevalence and correlation of gender-based violence among female university students in northern Nigeria. The authors administered 300 questionnaires. The study revealed a prevalence of 58.8% for GBV. GBV experienced by students included physical, sexual and emotional violence. In addition, religious affiliation, ethnicity, indigeneship, marital status, campus residence and faculty affiliation were essential predictors of GBV.

Finchilescu & Dugard (2018) assessed the experiences of GBV at the University of Wits. Their study found that in all facets of life, women were the highest sufferers of GBV with 74.2% of the students (178), 90.5% of the Administrative workers (19), and 84.3% of the Academic members of staff (43).

Gordon & Collins (2013) investigated the understanding of GBV among female students at a metropolitan tertiary institution in KwaZulu – Natal. Using unstructured interviews, they recruited 12 females who were resident of the university. They then used the technique of discourse analyses and analyzed the interviews conducted. It was found that the routine daily activities of these ladies involved in the research were crippled owing to the fear of becoming a victim of GBV.

Empirical evidence worldwide suggests the need to tackle gender-based violence to make our universities safe spaces for academic work to thrive and bring out more individuals willing to help transform their countries, especially females.



This model identifies the type and source of GBV. It could be intimate partner violence, verbal or emotional violence. It is necessary to find out the kind of violence so that the framework would be set rolling. When this is done, then the perpetrator, who is the source, comes in. Identifying the type of GBV is not enough; therefore, the need to determine the source of the violence to enhance control measures.

The causes are usually to find out why the perpetrator is performing such acts. For example, some rapists give a dozen reasons for their actions. The framework would determine the reasons behind the violence and whether there is a substance in the communicated reasons.

This policy framework again looks at laying down preventive measures, which would be explained in detail. The chunk of the work on GBV is to the menace, if not completely do away with it. This implies that our institutions as a country should work. At any point in time that there is the occurrence of GBV, adequate health care should be provided for the one who suffers it. This includes the government's initiative to offer free psychological or emotional care in our schools where students can go there for counselling and in our societies for everyone. The government could set up a medical bill platform for women who suffer physical harm, so they are not charged for medical care. The police need the required logistics to deal with cases of GBV at any point in time. The laws on GBV engage suitable attention, initiatives and solutions. The women and gender ministry should also consider teaming up with NGOs, traditional councils and other advocacy groups to deepen education on GBV.

Perpetrators must face sanctions. This part of the framework seeks to ensure justice and fairness such that those who mete out GBV are appropriately punished, at least to serve as a deterrent to people who would want to repeat the action. The current GBV crime control system fails to address this issue adequately and usually allow some people to escape punishment after episodes of GBV. This policy framework ensures stricter adherence to sanctioning or cautioning to be sanity in the system.

That notwithstanding, girl child education should be massively encouraged without any form of hindrance. This boosts the girl-child's confidence, and they find their voice in unison when things go wrong with the colleague. Women are described as a vulnerable gender. With the proper education, they would defend their rights and fight a suitable course of preventing GBV. Then institutions of the country, including government and private ones, should consider increasing the representation of women by appointing or voting for well-deserving women to hold some key positions. This serves as a powerful platform to help advocacy and thus prevent GBV.

Finally, the framework seeks to ensure governments commitment to preventing GBV. Once the government is committed to curbing the menace, the battle would be almost won. This stems from the fact that the necessary arrangements would be put in place to combat the threat, including the resources needed for it.

Conclusion

The fight against GBV is a colossal battle. The right strategies and approaches are essential to realizing the dream of a GBV - free world as proposed by the sustainable development goals. Nevertheless, some hindrances and challenges hinder the provision of sustainable solutions to GBV. GBV is on the rise globally and especially in Sub - Saharan Africa. However, massive improvement would be seen if the continent works hard. The government of any country should be committed to this honourable course and ensure a GBV- free world.

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